



Mental Health Clinical Incident Management

This procedure should be read in conjunction with [SMHS SAC1 clinical incident investigations. Roles and Responsibilities.](#)

1. Background

In WA, the Department of Health (DoH) [Clinical Incident Management Policy \(MP 0122/19\)](#) requires all clinical incidents classified as Severity Assessment Code 1 (SAC 1) to have comprehensive analysis or other analysis of similar rigorous methodology to identify possible contributing factors. This can include concise incident analysis and/or multi-incident analysis. These comprehensive reviews of care are required to be completed within 45 working days of the SAC 1 clinical incident being notified to the Patient Safety Surveillance Unit (PSSU). Further information about investigation methods can be found in the DoH [Clinical Incident Management Guideline](#) Section 6.4.

This document outlines the steps to be taken by SMHS MH staff involved directly or indirectly in incident management, including requirements, roles and responsibilities. This document must be used in conjunction with the Department of Health Clinical Incident Management policy, Guideline and [SAC1 clinical incident investigations Roles and responsibilities](#) which include the practices and principles of Restorative Just Learning Culture (RJLC).

1.1. About this document

SMHS MH recognises that clinical incidents including SAC 1's involving death or serious harm can be devastating to the wellbeing of patients, family/significant others and staff. This procedure aims to ensure the right steps are taken following a clinical incident to ensure staff are supported and reassured the 'service' takes responsibility for incident management, reinforcing investigations are not about finding fault but reviewing the care and treatment provided by the service for system issues which may have contributed to the incident. SMHS MH also recognises that all clinical incidents are an opportunity for reflection and learning. SMHS MH promotes a culture of reporting clinical incidents and has adopted a restorative just culture approach to safety, investigation, and improvement.

1.2. Key definitions

Term	Definition
Adverse Event	A clinical incident where an injury/harm is caused by medical management or complication thereof, instead of the underlying disease. It results in an increase in the level of care and/or prolonged hospitalisation and/or disability at the time of discharge.
Clinical incident or near miss	An event or circumstance resulting from healthcare provision (or lack thereof) which could have or did lead to unintended and/or unnecessary physical or psychological harm to a patient.
Comprehensive analysis	Analysis used for complicated and complex incidents that resulted in serious harm, or the significant risk thereof.
Mental Health Incident Review Meeting	SMHS MH Meeting responsible for: • confirming new SAC1 incidents • approving completed SAC1 clinical incident investigation reports. • approving evaluation reports • considering trends/themes • identifying service wide improvements • determining what is to be escalated to the SMHS MH Clinical Governance Committee.
Open disclosure	An open discussion with a patient about an incident(s) that resulted in harm to that patient while they were receiving health care. The elements of open disclosure are an apology or expression of regret (including the word 'sorry'), a factual explanation of what happened, an opportunity for the patient to relate their experience, and an explanation of the steps being taken to manage the event and prevent recurrence. Open disclosure is a discussion and an exchange of information that may take place over several meetings.
Restorative Just Learning Culture	A culture that acknowledges and supports all who have been impacted by critical events – patients, staff, families, and carers to heal, learn and improve following harm. This is a culture that identifies opportunities for systemic learning from systemic issues and focuses on safety, trust, fairness, learning and accountability.
SAC 1	A clinical incident that has or could have resulted in serious harm or death (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness.
SAC 2	A clinical incident that has or could have resulted in moderate harm (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness
SAC 3	A clinical incident that has or could have resulted in minor or no harm (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness.

SAC1 of concern

A significant clinical incident where the outcome of the investigation must be reviewed by the SMHS Area Executive Group and the Board Safety Quality Committee. Criteria for SAC 1s of concern includes but is not limited to:

1. Sentinel events.
2. Rare / uncommon events causing severe patient harm / death.
3. Extraordinary events requiring activation of the SMHS Serious Adverse Events Guideline
4. Repeat events for a current area of focus or concern outside of those undergoing a quality improvement (QI) project.
5. A cluster of events outside of those currently undergoing a QI project
6. Events likely to result in significant learning opportunities across SMHS and the broader health system.
7. Events where there is the potential for reputational damage.
8. Events posing medico-legal risk.
9. Events that are subject to Ministerial / media focus.

1.3 Mental Health Incident Review (MHIR)

The MHIR members are responsible for endorsing the clinical incident review report and any recommendations. By signing the report, MHIR are confirming that they support the findings of the review and are confident that the recommendations will address the contributing factors, or they support the request for declassification.

If MHIR are not supportive of clinical incident recommendations or the request for declassification, this decision with rationale must be provided to the SAC 1 panel members in writing together with an opportunity for the panel to respond. The final report must be endorsed by MHIR members and submitted to the PSSU within 45 working days of the date of notification to PSSU.

MHIR are responsible for reviewing and approving evaluation reports prior to submission to PSSU.

1.4 MHIR Membership

- Executive Director MH (Chair)
- Area Director Clinical Services
- Area Director of Nursing
- SMHS MH Manager Safety & Quality
- SMHS MH Nurse Director
- SMHS MH Deputy Director Clinical Services

In addition:

- Service Director/Co-Director of relevant site
- Medical Director (PaRK) or Head of Department (FSFH) of relevant site

2. Procedure

2.1. SAC1 Clinical Incident Investigation Requirements

Step	Timeframe	Responsibility	Actions
Identification	Immediate (day of incident or day incident reported)	Staff involved in incident	- Clinical incident occurs - take immediate action to reduce risk to the patient and staff. - Inform relevant senior management. - Document incident details in patients' medical record. - Enter incident into Datix CIMS as soon as practicable (within 48hrs).
		Line manager/relevant management/senior staff	- Identify open disclosure lead. - Commence 'Unexpected death of a MH consumer Checklist MR40.01 (where appropriate) - Involve Aboriginal Health Strategy/Aboriginal Mental Health Liaison if Aboriginal patient and / or staff
		Service Co-Director Head of Service	- Inform Executive Director of potential SAC1. - Refer to the Employee Assistance Program where indicated including access to Aboriginal counsellors / psychologists - Activate staff post critical incident support if required. Refer to SMHS Critical Incident Stress Management Procedure.
Notification	Short term 1 - 3 days	SQR and senior management/leadership	- Preliminary details of incident reviewed and discussed to establish next steps. - Complete a Serious Clinical Incident Notification (SCIN) - Initial open disclosure discussion. Refer to SMHS Open Disclosure Policy.
		MHIR meeting	Confirmation of SAC rating. Refer to Appendix A SMHS MH SAC decision tree.
	Short term 3 – 7 days	SQR and senior management/leadership	- Formal open disclosure process commenced. - Confirmation of SAC1 in Datix to be completed by SQR staff only. - PSSU notified within 7 working days of confirmation of SAC1 status.

			- For clinical incidents with potential to impact the wider system refer to the SMHS Management of Serious Adverse Events Guideline. 45 working day timeline commences from the date of notification to PSSU. - SCIN disseminated to relevant staff.
Incident review	<35 working days	SQR /SAC 1 Panel Chair	- SAC1 Panel Chair approved by HOD: - MHIR to determine SAC1 Panel Chair if incident is considered SAC 1 of concern. - Determine panel members, may include carer/consumer, education and external subject matter expert as appropriate. - Ideally all panel members will have attended clinical incident investigation training. - Determine appropriate SAC1 investigation methodology. - SQR or delegate complete detailed timeline (and diagrammatic of events if required) from medical record and PSOLIS. - Review of relevant best practice clinical literature and standards - Discussion with staff involved in incident to understand what occurred from their perspective. - Determine if the family wishes to contribute to the review and progress with the most relevant senior person as lead. - Initial SAC1 meeting to analyse course of events that may have led to the incident and identify any healthcare related contributing factors. More than one meeting may be required to complete the review. - Where healthcare related contributing factors are identified a meeting will then be scheduled with key stakeholders to review the draft SAC1 report and develop meaningful recommendations for any contributing factor(s) and allocate action owners. - SQR or delegate draft final report post meeting for review by the SAC1 panel.

Reporting	35-45 working days	MHIR meeting	- Review final SAC1 investigation report with recommendations. Where no systems issues were identified in the review and the incident considered to be not preventable approve/decline request for declassification. - MHIR approve or provide feedback as required.
		SQR and senior management/leadership	- Initial verbal feedback to staff involved by appropriate senior staff +/- SAC 1 Panel Chair - Final SAC1 report endorsed by the Executive Director or delegate. - Final SAC1 report provided to relevant site-based Executive for awareness. - Submitted to PSSU within 45 working days of notification
Feedback	>45 working days	Relevant Senior Staff	- Formal feedback provided to: - staff involved in the clinical incident. - to the team - to the service via business meetings - Feedback to the family if required. - SAC1 case presentations at site Morbidity & Mortality meetings - SAC 1 Clinical incident review summary made available to staff according to site-based process. Refer to Appendix B for example template.
Closing the loop	<6 months	SQR/MHIR meeting	- Action owners feedback completion of recommendations and evaluations to SQR. - Achievement of recommendations and evaluations monitored by SQR in relevant Incident and Service Improvement meetings. - Final implementation of recommendations, evaluated by MHIR and approved to close in DATIX. - SQR ensure recommendations are closed in DATIX. - Progress report detailing progress of recommendations to PSSU within 6 months (182 calendar days) of the investigation report submission.

			○ If not completed within 6 months a SAC1 evaluation of recommendations to be submitted to PSSU within 12 months (365 calendar days)
Review	Ongoing	SQR /Senior staff	• Review clinical risks regularly. • Thematic reviews as indicated. • Ongoing quality and service improvement activities

2.2. SAC 2 and SAC 3 Clinical Incident Investigation Requirement:

During the first 48 hours of notification, SAC 2 and 3 clinical incidents are to be reviewed by the relevant staff involved in the management of clinical incidents for the specific location. Relevant staff are to follow local processes to manage confirmed SAC 2 and SAC 3 clinical incidents. This includes but is not limited to:

- commence an initial investigation to identify process and system failures that may have led to the clinical incident occurring.
- implement preliminary actions to mitigate further risk of harm to the patient and/or staff.
- initiate an appropriate level of open disclosure processes.
- **SAC2 and SAC3** investigation completed within **60 working days**.
- Final sign off **SAC2** by HoD and should include relevant recommendations or reference to quality improvement initiatives.
- Final sign off **SAC3** by local processes.

3. Compliance Monitoring

Compliance with the DoH Clinical Incident Management policy reporting requirements and timeframes for SAC1 clinical incident investigations will be monitored monthly by the SMHS MH Clinical Governance Committee.

Compliance with the DoH Clinical Incident Management policy timeframes for SAC 2 & 3 clinical incident investigations will be monitored monthly by the site Clinical Governance Sub Committees.

4. Legislation

- [*Mental Health Act 2014*](#)
- [*Health Services Act 2016*](#)
- [*Public Sector Management Act 1994*](#)

5. Related Documents

5.1. Department of Health policies

- [MP 0122/19 Clinical Incident Management Policy](#)
- [Clinical Incident Management Guideline](#)
- [MP 0152/21 Information Management Governance Policy](#)
- [MP 0098/18 Review of Death](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)

5.2. SMHS policies

- [SAC1 clinical incident investigations Roles and responsibilities](#)
- [Open Disclosure Policy](#).

- [Morbidity & Mortality Review Standard](#)
- [Risk Management](#)
- [Critical Incident Stress Management](#)

6. Document Control

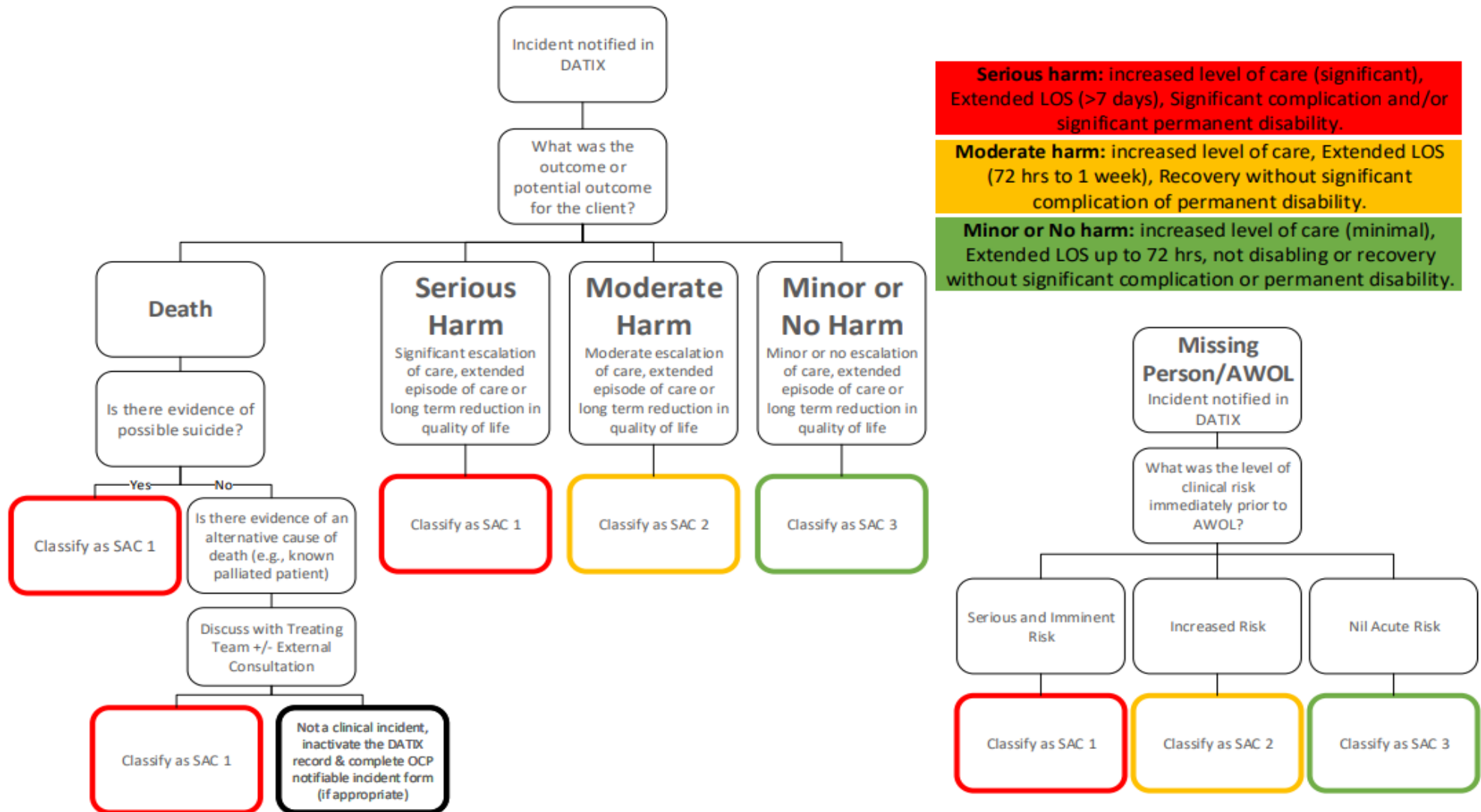
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GP 1.0	29 August 2025	15 August 2025	15 August 2028	August 2028	Original version

6.1. Approval

Approval by	SMHS MH Policy Committee
Approval date	12 August 2025

Appendix A: Mental Health SAC Decision Tree

Mental Health SAC Decision Tree



Appendix B: SAC1 Staff Summary



Government of Western Australia
South Metropolitan Health Service

Clinical Incident Review Summary

Clinical incidents are categorised using Severity Assessment Code (SAC) ratings to determine the appropriate level of analysis, action and escalation required.

SAC1 – A clinical incident that has or could have resulted in serious harm or death (including near miss incidents); and which is attributed to health care provision (or lack thereof) rather than the patient's underlying condition or illness.

SAC1 Review: Date XX/XX/XX Category:.....

Summary of incident

Investigation Panel Findings

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Review Outcomes

Add in additional information/resources:

